

Week 7 — Diagnosis by Inquiry

EAM310 · EAST ASIAN MEDICINE DIAGNOSIS I · VUIM SUMMER 2026

⚠ Quiz 5 — August 28, 2026

Class: Friday August 28, 2026 · 9AM–1PM

Prof. Daniel Schrier, D.O.M., L.Ac.

Jonathan Centeno, D.AcHM Candidate

REQUIRED READINGS — DUE BEFORE AUGUST 28

FCM CHAPTER 24 — MACIOCIA

- Diagnosis by Interrogation — full chapter
- Two aspects: General vs Specific interrogation
- Nature of "symptoms" in Chinese medicine
- The 16 Questions — full system, all categories
- Terminology problems: Chinese → Western patient language

CAM CHAPTER 9 — CHENG XINNONG

- Inquiry (問診) as third of Four Methods
- Classical Ten-Question Song (十問歌)
- Zhang Jingyue's original 10-question framework
- Modern adaptation for clinical practice
- Integration with the other 3 diagnostic pillars

WEEK 7 LEARNING OBJECTIVES

- Distinguish the General vs Specific aspects of interrogation
- Recite the 16 Questions in Maciocia's revised order
- Differentiate Full-Heat vs Yin-deficiency thirst presentations
- Interpret stool/urine findings for Cold/Heat/Full/Empty
- Explain why TCM "symptoms" are broader than Western signs/symptoms
- Explain why Maciocia moved Pain to #1 and Chills/Fever to #12
- Interpret sweating by area, timing, and quality
- Apply Maciocia's 8-step full intake procedure to a case

ASSESSMENT CALENDAR

Date	Event	Material Covered
Aug 21	Week 6 Class	Diagnosis by Observation
Aug 28	⚠ Quiz 5	Diagnosis by Inquiry — 16 Questions, General/Specific interrogation
Sep 4	Quiz 6	Week 8 material — Inquiry cont. + Palpation intro
Sep 11	FINAL + Journal 26–50	Weeks 6–9 comprehensive

IN-CLASS PRACTICE — WHAT TO EXPECT

INTERROGATION PRACTICE LAB

Week 7 includes partner interview practice. Come ready to:

- Conduct a mock intake using Maciocia's 8-step procedure on a partner
- Practice translating Chinese-derived symptom language into Western patient-friendly phrasing
- Ask at least 3 questions from each of the first 9 categories (Pain → Sleep)
- Document findings in your Pulse/Tongue Journal, including the cause you uncovered

TWO ASPECTS OF INTERROGATION 問診	
General	Talk between doctor and patient about how the problem arose, living/working conditions, emotional and family environment. Aim: find the cause , not the pattern.
Specific	Aims at identifying the prevailing pattern(s) of disharmony — by Internal Organs, by channels, by Four Levels, etc. Aim: find the pattern .
Maciocia's own practice: "I personally concentrate first on the clinical manifestations to identify the patterns involved. It is only after I have come to a conclusion about the patterns involved that I ask general questions about lifestyle, work, emotions... to try and find the cause." Pattern first, cause second — don't blur the two.	

"SYMPTOMS" IN CHINESE MEDICINE — BROADER THAN THE WEST

Western medicine: symptoms = subjective/objective manifestations of a disease process. Chinese medicine: symptoms are ANY manifestation that forms part of the whole clinical picture — many are unrelated to a definable "disease."

"Non-Western" Symptom	What It Points To
Absence of thirst	Confirms a Cold condition
Incapacity to make decisions	Deficiency of the Gall Bladder
Dislike of speaking	Deficiency of the Lungs
Propensity to outbursts of anger	Rising Liver-Yang or Liver-Fire
Desire to lie down	Weakness of the Spleen
Dull appearance of the eyes	Disturbance of the Mind, emotional problems
Deep midline crack on the tongue	Propensity to deep emotional problems

TERMINOLOGY PROBLEM — CHINESE → WESTERN PATIENT LANGUAGE

Chinese patients spontaneously describe a "distending pain." An English-speaking Western patient instead says "bloated" or "bursting." Learn to translate — the pattern logic doesn't change, but the vocabulary does.

MACIOCIA'S 8-STEP FULL INTAKE PROCEDURE	
1	Let the patient speak freely about the presenting problem
2	Ask specific questions to determine the pattern, establishing onset
3	Ask questions to exclude or confirm other patterns
4	Broadly follow the 16 Questions
5	Ask about past diseases and operations
6	Look at the tongue, feel the pulse
7	Ask about family illnesses
8	Inquire about emotional/family/work life to find the cause
Golden rule: "What matters is the general PICTURE formed by the symptoms... it is only for didactic purposes that we list symptoms in isolation." Never diagnose off one isolated answer — always integrate the full clinical picture.	

THE CLASSICAL TEN-QUESTION SONG 十問歌 (ZHANG JINGYUE)			
#	Classical Question	#	Classical Question
1	Cold and heat 寒热	6	Chest and abdomen 胸腹
2	Sweat 汗	7	Deafness/hearing 聋
3	Head and body 头身	8	Thirst 渴
4	Stool and urine 二便	9	Old disease 旧病
5	Food and drink 饮食	10	Cause 因

10 → 16, what actually changed: Maciocia didn't discard Zhang Jingyue's classical 10 — he split compound categories apart (Chest/Abdomen, Head/Body become distinct), promoted implicit categories to full standalone items (Energy, Emotional, Sexual, Women's, Children's), then reordered so Pain leads and Chills/Fever confirms at the end. Same tradition, modernized packaging.

Why the order changed: Maciocia moved Pain to #1 (Western patients' most common complaint) and Chills/Fever to #12, only asked at the end to CONFIRM the Hot/Cold nature of an already-identified pattern. The classical Qing-dynasty 10 questions assumed acute exterior febrile disease (Shang Han Lun bias).

THE 16 QUESTIONS — MACIOCIA'S ORDER

#	Area	#	Area
1	Pain	9	Sleep
2	Food and taste	10	Sweating
3	Stools and urine	11	Ears and eyes
4	Thirst and drink	12	Cold, heat, fever
5	Energy levels	13	Emotional symptoms
6	Head, face, body	14	Sexual symptoms
7	Chest and abdomen	15	Women's symptoms
8	Limbs	16	Children's symptoms

MASTER PAIN TABLE — FULL VS EMPTY, COLD VS HEAT

Factor	Empty	Full	Cold	Heat
Pressure	Alleviated	Aggravated	—	—
Type	Dull, lingering	Sharp	Cramping	Burning
Temperature	—	—	Better w/ heat	Better w/ cold
Onset	Slow, gradual	Sudden	—	—
Rest/movement	Better w/ rest	Better w/ movement	Better w/ movement	Worse w/ movement

Cause	Character	Location
Qi stagnation	Distension > pain, no fixed location	Epigastrium, abdomen
Blood stasis	Boring/stabbing, FIXED	Head, chest, epigastrium, Uterus
Cold	Cramping, better with heat	Epigastrium, Uterus, joints
Damp-Heat	Burning + fullness/heaviness	Forehead, epigastrium, Bladder, joints
External Wind	—	Occiput

Q4 — THIRST QUICK REFERENCE

Large cold drinks	Full-Heat (any organ)
Small sips	Yin deficiency (ST/KD)
No thirst	Cold (ST/SP)
Thirst, no desire to drink	Damp-Heat
Desire cold liquids	Heat pattern
Desire warm liquids	Cold pattern

Q3 — URINE QUICK REFERENCE

Enuresis/incontinence	Kidney deficiency
Frequent + copious	Kidney-Yang deficiency
Frequent + scanty	Qi deficiency
Scanty, dark	Kidney-Yin deficiency
Pain before / during / after	Qi stag / Damp-Heat BL / Qi def
Turbid, cloudy	Dampness in Bladder

Q3 — STOOLS QUICK REFERENCE

Constipation, worse after BM	Empty pattern
Improves after BM	Full pattern
Constipation + thirst + yellow coat	Heat in Stomach/Intestines
Constipation, elderly/postpartum	Blood deficiency
Goat-stool constipation	Liver-Qi stagnation + Intestinal Heat
Chronic loose stools/diarrhoea	Spleen and/or Kidney-Yang deficiency
Diarrhoea every early morning	Kidney-Yang def ("cock-crow diarrhoea")
Diarrhoea + mucus	Dampness in Intestines
Diarrhoea + mucus + blood	Damp-Heat in Intestines
Black or very dark stools	Stasis of Blood
Borborygmi + loose stools	Spleen deficiency
Borborygmi + distension, no loose stools	Liver-Qi stagnation
Foul-smelling stools	Heat
Absence of smell	Cold

Q2 — FOOD & TASTE

Finding	Pattern
Relieved by eating	Empty pattern
Aggravated by eating	Full pattern
Lack of appetite	Spleen-Qi deficiency
Excessive hunger	Stomach-Heat
Fullness after eating	Retention of food or Dampness
Distension	Qi stagnation
Sharp pain	Blood stasis
Heaviness	Dampness
Preference for hot food	Cold pattern
Preference for cold food	Heat pattern
Taste	Pattern
Bitter	Full-Heat, Liver or Heart (constant = LV-Fire; only after sleepless night = HT-Fire)
Sweet	Spleen deficiency or Damp-Heat
Sour	Retention of food, or Liver invading Stomach
Salty	Kidney-Yin deficiency
Pungent	Lung-Heat
Lack of taste sensation	Spleen and Stomach deficiency

Q5 — ENERGY LEVELS (TIREDNESS)

Not one of the classical 10 — Maciocia added it because tiredness is one of the most common Western complaints (~12% of his patients sought treatment for tiredness alone). Ask early, as soon as a Deficiency pattern is suspected. **Remember:** tiredness is not always Deficiency — it can be Full (Dampness, Phlegm, Qi stagnation), especially with a Slippery or Wiry pulse.

Chronic Tiredness + ...	Pattern
Desire to lie down, poor appetite, loose stools	Spleen-Qi or Spleen-Yang deficiency
Weak voice, catches colds easily	Lung-Qi or Lung-Yang deficiency
Backache, feeling cold, frequent urination	Kidney-Yang deficiency
Dizziness, blurred vision, scanty periods	Liver-Blood deficiency
Anxiety, dry mouth at night, no tongue coating	Kidney-Yin deficiency
Feeling of heaviness	Dampness
Heaviness + muzziness + dizziness	Phlegm
Abdominal distension, irritability, Wiry pulse	Liver-Qi stagnation
Alternating hot/cold, unilateral tongue coat, Wiry pulse	Lesser Yang (Shaoyang) pattern

Q6 — HEAD, FACE & BODY: HEADACHE

Onset/Timing	Pattern	Location	Channel / Cause
Recent, short duration	Exterior Wind-Cold	Nape of neck	Greater Yang (ext. Wind-Cold or int. KD def)
Gradual, in attacks	Interior type	Forehead	Bright Yang (ST-Heat or Blood def)
Daytime	Qi or Yang deficiency	Temples/sides	Lesser Yang (ext. Wind or LV/GB Fire)
Evening	Blood or Yin deficiency	Vertex	Terminal Yin (usu. LV-Blood def)
Night-time	Blood stasis	Whole head	Exterior Wind-Cold
Character / Condition	Pattern		
Heavy feeling	Dampness or Phlegm		
Pain "inside," "hurting the brain"	Kidney deficiency		
Distending, throbbing	Liver-Yang rising		
Boring, like a nail	Blood stasis		
Aggravated by fatigue, better with rest	Qi deficiency		
Aggravated by emotional tension	Liver-Yang rising		
Aggravated by sexual activity	Kidney deficiency		

Dizziness quick reference: Severe giddiness/loss of balance = internal Wind · With throbbing headache = Liver-Yang rising · With heaviness/muzziness = Phlegm obstructing.

Q7 — CHEST & ABDOMEN

Stabbing chest pain	Stasis of Blood in the Heart
Chest pain + cough + yellow sputum	Lung-Heat
Oppression of the chest	Phlegm in Lungs, or severe LV-Qi stagnation
Hypochondrium distension	Liver-Qi stagnation
Hypochondrium stabbing pain	Liver-Blood stasis
Epigastric pain, dull, better w/ eating	Empty type (ST-Qi def or Empty-Cold)
Epigastric pain, severe, worse w/ eating	Full type (food retention, ST-Heat, Damp-Heat, Blood stasis, LV invading ST)
Epigastric fullness	Dampness, or slight SP-Qi deficiency
Epigastric distension	Liver-Qi stagnation
Lower abdominal pain	Interior Cold, LV-Qi/Blood stagnation, Damp-Heat, or Blood stasis in Intestines/Uterus
Abdominal pain relieved by BM	Full nature
Abdominal pain worse after BM	Empty nature
Hypogastric pain	Damp-Heat in Bladder, or LV-Fire infusing down

Palpation cross-check: A mass that MOVES under the fingers = Qi stagnation. A FIXED, hard mass = Blood stasis. Tenderness at Ren-17 (Shanzhong) on light pressure = classic Heart-Blood stasis sign.

Q8 — LIMBS & BODY

Only routinely asked about when the patient presents with a specific limb complaint — **except** always ask about numbness with Blood deficiency, and heaviness with Dampness.

Numbness, both sides (arms+legs or hands+feet)	Blood deficiency
Numbness, ONE side only (fingers/elbow/arm)	⚠ Internal Wind + Phlegm — possible impending Wind-stroke
Joint pain, wandering	Wind
Joint pain, fixed, very painful	Cold
Joint pain, fixed + swelling + numbness	Dampness
Joint swelling + redness	Damp-Heat
Severe stabbing pain + rigidity	Blood stasis
Backache, continuous, dull, better w/ rest	Kidney deficiency
Backache, recent onset, severe, stiff	Sprain causing Blood stasis
Backache, worse in cold/damp weather	Exterior Cold-Damp invading back channels
Backache, boring, can't turn waist	Blood stasis

Q9 — SLEEP & Q LETHARGY QUICK REFERENCE

Sleepy after eating	Spleen-Qi deficiency
General lethargy + heaviness	Retention of Dampness
Lethargy + heaviness + dizziness	Phlegm
Extreme lethargy + cold feeling	Kidney-Yang deficiency
Insomnia, dream-disturbed	Heart Blood deficiency
Difficulty falling asleep, restless	Heart Yin deficiency / Empty-Heat
Lethargic stupor + rattling throat + Slippery pulse	Phlegm blurring the Mind

Q10 — SWEATING: FOUR FACTORS TO EVALUATE

Factor	Finding	Meaning
AREA OF BODY		
Area	Only on head	Heat in the Stomach, or Damp-Heat
	Oily sweat on forehead	Collapse of Yang
	Only arms and legs	Stomach and Spleen deficiency
	Only on hands	Lung-Qi or Heart-Qi deficiency
	Whole body	Lung-Qi deficiency
	Palms, soles, chest ("five-palm sweat")	Yin deficiency
TIME OF DAY		
Timing	Daytime	Yang deficiency
	Night-time	Yin deficiency (occasionally Damp-Heat)
CONDITION OF ILLNESS		
Severity	Profuse cold sweat during severe illness	Collapse of Yang
	Oily sweat on forehead "like pearls," not flowing	Collapse of Yang — imminent danger
QUALITY OF SWEAT		
Quality	Oily	Severe Yang deficiency
	Yellow	Damp-Heat

Q11 — EARS & EYES

Tinnitus, sudden/loud	Full (LV-Fire, LV-Yang rising)
Tinnitus, gradual/faint	Empty (KD-Essence def)
Blurred vision	Liver-Blood deficiency
Dry eyes	Liver-Blood or Yin deficiency
Floaters	Liver-Blood or KD-Essence def

Q12 — COLD/HEAT/FEVER (CONFIRMS, NOT LEADS)

Aversion to cold, no sweat	Exterior Full (Wind-Cold)
Aversion to wind, some sweat	Exterior Empty (Wind, Upright Qi def)
Alternating chills & fever	Shaoyang / half-ext-half-int
Tidal fever (afternoon)	Yin deficiency / Empty-Heat
Aversion to cold, desires warmth	Interior Cold (Yang def)

Clinical logic: Q12 is placed near the END of the 16 Questions deliberately — by this point you likely already suspect Hot or Cold from pain, thirst, stools, and sweat findings. Cold/Heat/Fever questions *confirm* that suspicion; they rarely lead the diagnosis in modern Western practice the way they did in Shang Han Lun-era febrile disease.

Q13 — EMOTIONAL SYMPTOMS		Q14 — SEXUAL SYMPTOMS	
Depression, sighing	Liver-Qi stagnation	Ask gently, following the conversation — often skipped by modern Chinese doctors due to cultural prudery, but Maciocia insists it always be included.	
Irritability, anger outbursts	LV-Yang rising / LV-Fire		
Anxiety, worry	Heart/Spleen involvement	Low libido	KD-Yang or Essence deficiency
Fear, timidity	Kidney involvement	Premature ejaculation	KD-Qi not consolidating, or Heat
Excessive grief	Lung involvement	Intercourse during/soon after period	Qi/Blood stasis in Uterus (per Fu Qing-Zhu)
Q15 — WOMEN'S SYMPTOMS: MENSTRUATION			
Parameter	Finding	Pattern	
CYCLE			
Timing	Always early	Blood-Heat or Qi deficiency	
	Always late	Blood deficiency, stagnation, or Cold	
	Irregular (early & late)	LV-Qi/Blood stagnation, or Spleen def	
AMOUNT & COLOUR			
Amount/Colour	Heavy loss	Blood-Heat or Qi deficiency	
	Scanty	Blood deficiency, stagnation, or Cold	
	Dark-red / bright-red	Blood-Heat	
	Purple/blackish with clots	Blood stasis or Cold	
PAIN TIMING			
Pain	Before periods	Stagnation of Qi or Blood	
	During periods	Blood-Heat or stagnation of Cold	
	After periods	Blood deficiency	
Leucorrhoea	Finding	Pattern	
Colour	White	Cold, SP/KD-Yang def, or ext. Cold-Damp	
	Yellow	Damp-Heat in Lower Burner	
	Greenish	Damp-Heat in Liver channel	
Q16 — CHILDREN'S SYMPTOMS (ASK CAREGIVER + DIRECT OBSERVATION)			
Appetite, energy, sleep		General Qi/constitution baseline	
Behavior/mood changes		Screen for Liver/Heart disharmony early	
Digestive symptoms		Spleen system especially vulnerable in children	
Respiratory symptoms, earache		Lung/Kidney — children's Wei Qi still developing	

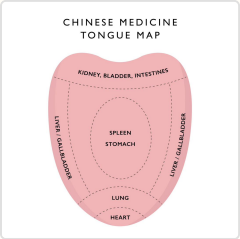
PATTERN NAME	DISEASE CATEGORY	WEEK / QUIZ

ETIOLOGY	ORGAN SYSTEM	DATE

CLINICAL ESSENTIALS — CASE NOTES

INQUIRY 問診 · KEY FINDINGS

CHINESE MEDICINE TONGUE MAP



PAIN / THIRST / SWEAT / SLEEP / CAUSE

PULSE 脈診 · CUN · GUAN · CHI

Wrist	CUN 寸	GUAN 關	CHI 尺
L HT/SI		LV/GB	KD/BL
R LU/LI		SP/ST	PC/TB

QUALITY / OVERALL

TREATMENT PRINCIPLE 治則

KEY POINTS / HERBS

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